

Learning Objectives:

1. Basic Well Child Visit Interviewing/History Taking
2. Growth
3. AAP recommendations
4. Psych Screenings
5. Vaccine Schedule
6. Likely pathologies
7. Scoliosis testing
8. Tanner Staging
9. Resources

Well child visits:

Ask about school, grades

Ask about screen time

Ask about Diet/Exercise

Ask about activities, sports participation, etc

Establishing good sleeping habits (sleeping about 9 hours a night)

Dentist visits, brushing twice a day!

Visual screening – need for glasses?

AAP recommends **1 random screening between 9-11 years** of age of lipids

Cardiac screening questions (LOC while exercising, chest pain, early cardiac death in family)

Obesity (BMI %tile over 95 for age and sex) is a growing problem:

1 in 5 US children and adolescents suffer from obesity



Keep an eye on length as well

Short Stature:

Constitutional Growth Delay

Familial

Pathologic

Idiopathic

Constitutional delay: catch up growth will occur,
bone age young

Familial: calculating mid-parental height

Pathologic: GH deficiency, malnutrition causes
(weight will also be affected)

Idiopathic: tests normal

Table 1. Formulas for Calculating Midparental Height in Children

Girls

$$[\text{Paternal height (cm)} - 13 \text{ cm} + \text{maternal height (cm)}] \div 2$$

or

$$[\text{Paternal height (in)} - 5 \text{ in} + \text{maternal height (in)}] \div 2$$

Boys

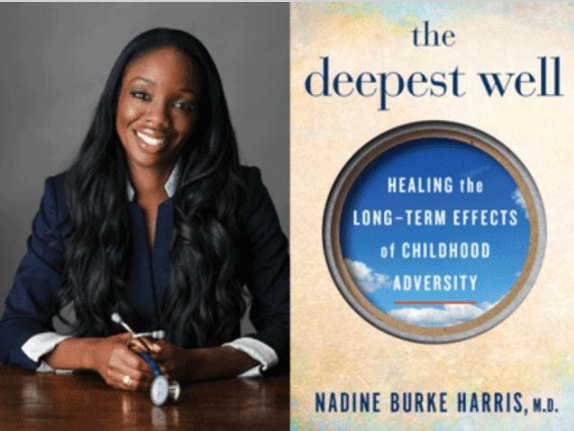
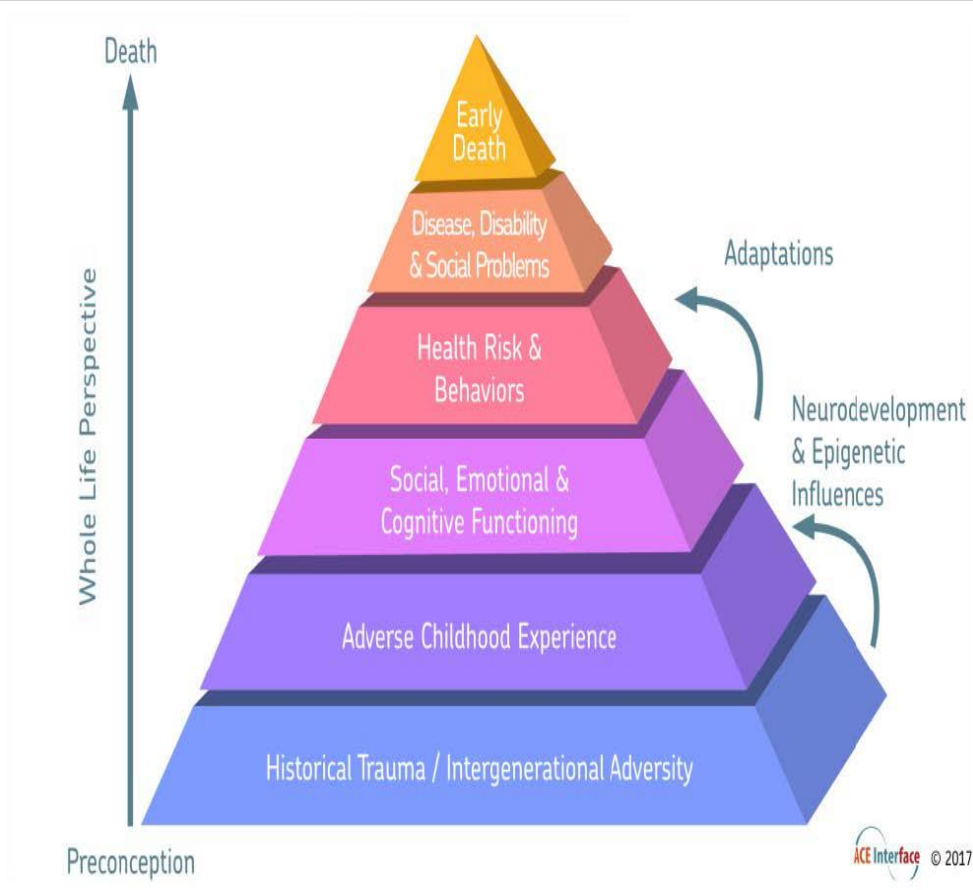
$$[\text{Paternal height (cm)} + 13 \text{ cm} + \text{maternal height (cm)}] \div 2$$

or

$$[\text{Paternal height (in)} + 5 \text{ in} + \text{maternal height (in)}] \div 2$$



Adverse Childhood Experiences (ACE) screening



ADHD (attention deficit hyperactivity disorder): is a common, chronic, pervasive, neurodevelopmental disorder characterized by developmentally inappropriate levels of hyperactivity, impulsivity, and inattention that adversely affect behavioral, emotional, cognitive, academic, occupational, and social functions

We use a Vanderbilt questionnaire to assess whether similar behaviors in 2 or more settings (school, home, after school place, work, play)

Inattention: more common in F, only inattention criteria met

Hyperactivity: more common in M, only hyperactivity criteria met

Combined: more common in M, both

At least several inattentive or hyperactive-impulsive symptoms occur **before age 12 years**

Strong genetic component but multifactorial

Hyperactivity/impulsivity symptoms usually appear by age 4 years and peak by age 7 or 8 years

Prominent symptoms are motor restlessness and disruptive behavior

Inattention symptoms usually appear by age 8 or 9 years

Older kids: usually inattention, hyperactivity is less pronounced

Can be mild, moderate to severe based on degree of impairment

Rule out: age appropriate behavior, anxiety/depression, learning disabilities, sleep disturbances, chronic illness, substance use, bipolar

Treatment: multi-modal (stimulant/nonstimulant medications plus behavior therapy) most beneficial – IEP (individualized education program) – Multimodal

In this age range, likely pathologies

Strep pharyngitis

peak incidence age 5-9

Use of Centor Criteria

Swab: Rapid Strep + Culture

Treatment: Beta-lactams, Amoxil

vs viral pharyngitis: supportive

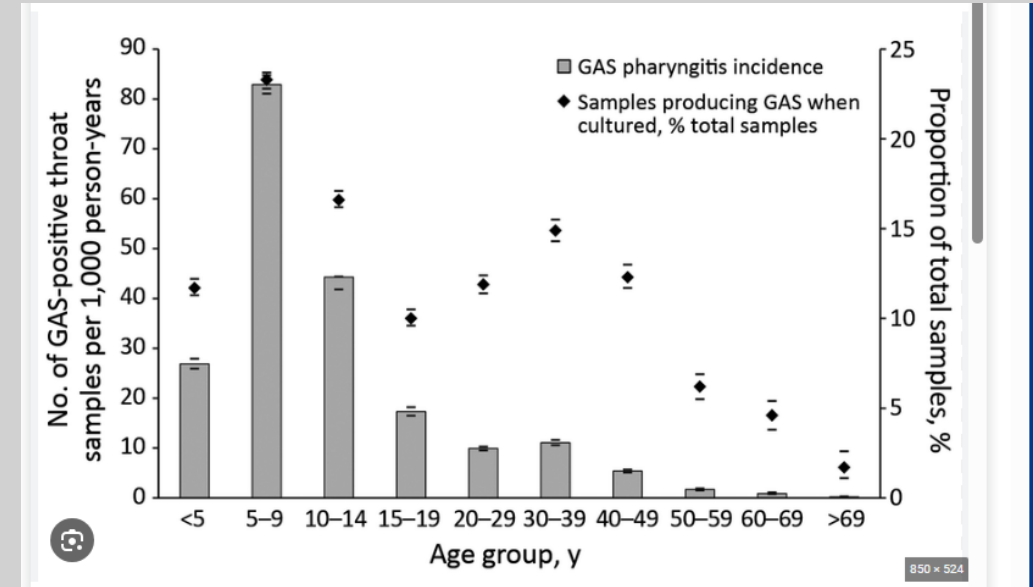


Table 1. Modified/McIsaac Centor Criteria³⁴

Criteria	Points
Age 3-14 years	1
Tonsillar swelling or exudates	1
Tender/swollen anterior cervical lymph nodes	1
Fever (temperature > 38°C [100.4°F])	1
Absence of cough	1
Total	_____

0 or 1 point = Group A *Streptococcus* (GAS) testing not usually required.

≥ 2 points = GAS testing usually required.



In this age range, likely pathologies

- During oral exam:
Make sure you look at dentition
- Getting adult teeth
- Prescence
of Dental caries

Tooth Types and Age of Eruption		
Approximate Age of Eruption ^[53]		
Tooth Type	Primary (months)	Permanent (years)
Central incisor	5–8	6–8
		Delayed tooth eruption can result from genetic disorders or systemic diseases.
Lateral incisor	5–11	7–9
Cuspids	24–30	11–12
First bicuspid	—	10–12
Second bicuspid	—	10–12
First molars	16–20	6–7
Second molars	24–30	11–13
Third molars	—	17–22
Source: Lunt RC, Law DB. A review of the chronology of eruption of deciduous teeth. <i>J Am Dent Assoc.</i> 1974;89(4):872–879.		



Abnormalities of the Teeth, Pharynx, and Neck

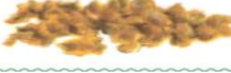
		Dental Caries Dental caries is a major global health and pediatric problem. White spots on the teeth often reflect early caries. The photographs show different characteristics of caries.
	Staining of the Teeth Various causes can lead to staining of the teeth of children, including intrinsic stains such as tetracycline (left) or extrinsic stains such as poor oral hygiene (cariou lesions shown in previous figures). Extrinsic stains can be removed.	

In this age range, likely pathologies

Constipation

- Very common in school aged children
- Withholding behaviors cause back up – rectal distension
- Associated with UTI as well
- Bristol score
- Examine abdomen: can feel stool in LLQ at times
- Ask about elimination
- KUB abdominal film (kidney ureters bladder xray) at times helpful
- Tx: increase fiber rich diet, miralax
- Not to be confused with appendicitis symptoms! - usually in older patients but cannot rule out – EXAM SKILLS MATTER!



THE BRISTOL STOOL FORM SCALE (for children) choose your POO!		
type 1		looks like: rabbit droppings Separate hard lumps, like nuts (hard to pass)
type 2		looks like: bunch of grapes Sausage-shaped but lumpy
type 3		looks like: corn on cob Like a sausage but with cracks on its surface
type 4		looks like: sausage Like a sausage or snake, smooth and soft
type 5		looks like: chicken nuggets Soft blobs with clear-cut edges (passed easily)
type 6		looks like: porridge Fluffy pieces with ragged edges, a mushy stool
type 7		looks like: gravy Watery, no solid pieces ENTIRELY LIQUID

In this age range, likely pathologies

Scoliosis Check

Adams Forward Bend test

Onset variable

Most found in adolescents greater than or equal to 11 yr)

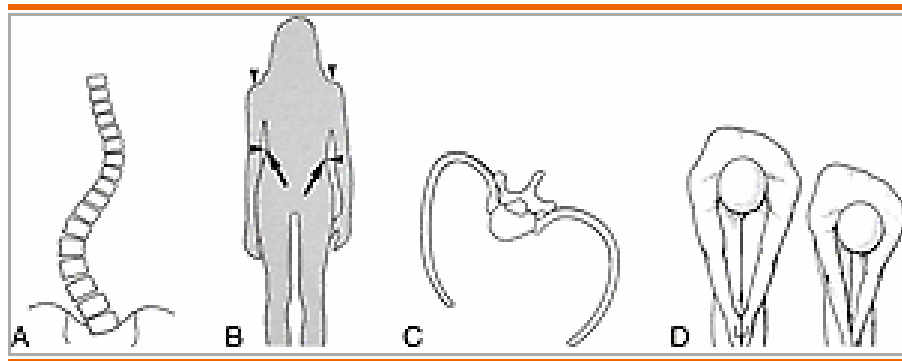
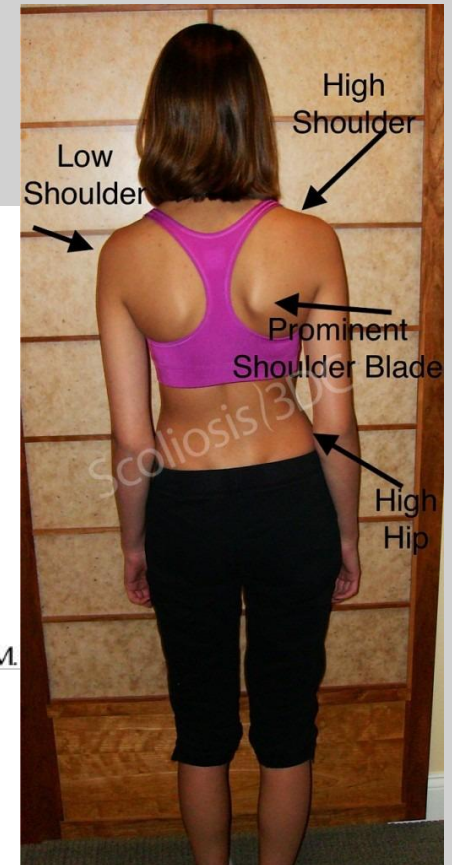


FIG. E1

Structural changes in idiopathic scoliosis.

Tucker Callanan, MD; Eren O. Kuris, MD

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In this age range, likely pathologies

Tanner Staging

Precocious puberty (central vs peripheral)

- All girls younger than 6 years of age with breast or pubic hair development
- Girls younger than 8 years of age with both breast and pubic hair development
- Girls younger than 8 years of age with only breast or pubic hair development: careful history and examination with a bone age determination and close follow-up
- Isolated premature thelarche or premature adrenarche – can be benign
- Boys: onset of secondary sexual characteristics in a boy before age 9

Delayed puberty will be discussed during adolescent lecture

